

OREGON STATE HOSPITAL

POLICY

SECTION 6:	Patient Care	POLICY: 6.067
SUBJECT:	Opioid Prescription Use and Opioid Overdose Reversal with Naloxone (Narcan®)	
POINT PERSON:	Deputy Chief Medical Officer	
APPROVED:	Sara Walker, MD  Interim Superintendent	DATE: OCTOBER 15, 2024
SELECT ONE:	☒ New policy ☐ Minor/technical revision of existing policy ☐ Reaffirmation of existing policy ☐ Major revision of existing policy	

I. PURPOSE AND APPLICABILITY

- A. This policy outlines the responsibility and expectation for Oregon State Hospital (OSH) staff in the use of opioids in treating chronic pain, to provide and monitor treatment for opioid use disorder with medication assisted treatment (MAT), and establishes expectations for staff to perform emergency reversal of suspected opioid-related overdose. The policy supports OSH's capability to deliver effective health care to its patients by:
1. offering patient-centered care, evidence-based care, harm reduction, linkage to treatment, and discharge with naloxone or prescription for naloxone when indicated;
 2. reducing harm of substance use at OSH;
 3. establishing a standing order for naloxone distribution and administration; and
 4. maximizing the safety of the patients, staff, volunteers, and visitors.
- B. This policy applies to all staff.

II. POLICY

- A. OSH practices a patient-centered treatment approach for substance use disorders (SUD) with the overall goal of reducing harm.
- B. OSH is committed to screening and identifying patients with SUD. The admitting psychiatric practitioner must screen for SUD and note their findings in the psychiatric admission assessment. The medical provider completing the initial medical history and physical examination must also screen for SUD and note their findings in the medical history and physical.

- C. Patients who screen positive for opioid use disorder may be offered evidence-based medications. MAT must be provided as required by the Medication-Assisted Treatment for Opioid Use Disorder Clinical Practice Guideline.
- D. Patients receiving long-term opioids for any reason (including, but not limited to, opioid use disorder and chronic pain management) are required to sign the Informed Consent and Management Agreement for Long Term Opioid Use.
- E. In cases of suspected opioid overdose of any individual on OSH property, the responder must first call a code blue. The responder does not need a verbal or written order from a prescriber to administer naloxone to a person suspected of experiencing an overdose. Attachment A: Standing Order for the Possession, Distribution, and Administration of Naloxone provides authorization for non-prescribers to administer naloxone in the event of a suspected overdose without an order specific to the person receiving the naloxone. Attachment A also contains the procedures for responding to opioid overdoses inside the secure perimeter and the procedures for responding to opioid overdoses outside the secure perimeter.
 - 1. All staff are required to complete training and policy attestation annually via online video and quiz on the recognition of opioid overdose and the administration of naloxone.
 - 2. Staff are not permitted to carry naloxone on off-grounds outings. If any staff or patient on an off-grounds outing appears to be experiencing an opioid overdose, staff must call 911.
 - 3. Staff must only use hospital-issued naloxone.
- F. OSH must provide and maintain on-site access to opioid overdose reversal medication naloxone (Narcan®) in various locations throughout the campus grounds, both within the secure perimeter and non-secure perimeter.
 - 1. Stock naloxone (Narcan®) must be clearly labeled, stored in original packaging, and stored in the following areas:

Junction City Campus	Salem Campus
Code Blue Carts throughout campus	Code Blue Carts throughout campus
Code Blue Bags throughout campus	Code Blue bags throughout campus and cottages
Automated dispensing cabinets (ADCs) on the units, in the Medical Clinic, and in the Treatment Malls	Automated dispensing cabinets (ADCs) on the units, in the Medical Clinic, and in the Treatment Malls
E-Kit in the room where patient mail is inspected	E-Kits in Dental Clinic, Vaccine Nurse's possession, Infection Prevention, and in the room where patient mail is inspected

2. Inside the secure perimeter, any Registered Nurse (RN), physician, or nurse practitioner (NP) may administer an opioid overdose reversal medication to treat any patient, staff, or member of the public suspected of having an opioid-related drug overdose, whether or not there is a known previous history of opioid use disorder.
 - a. A Licensed Practical Nurse (LPN) may administer an opioid overdose reversal medication at the direction of an RN.
 3. In areas that are inside an OSH building, but outside the secure perimeter, any Registered Nurse (RN), physician, or nurse practitioner (NP) may administer an opioid overdose reversal medication to treat any patient, staff, or member of the public suspected of having an opioid-related drug overdose, whether or not there is a known previous history of opioid use disorder.
 - a. A Licensed Practical Nurse (LPN) may administer an opioid overdose reversal medication at the direction of an RN.
 - b. On the Salem Campus these areas include the Kirkbride building, Facilities area, and Main Lobby. On the Junction City Campus these areas include the Administrative Area and Main Lobby.
 4. Outside the secure perimeter, any staff member or layperson may administer an opioid overdose reversal medication to treat any patient, staff, or member of the public suspected of having an opioid-related drug overdose, whether or not there is a known previous history of opioid use disorder.
- G. The Pharmacy Department must conduct periodic audits of the inventory of naloxone for upcoming expirations, and proactively remove and replace expired products according to Pharmacy protocol 33.6 Medication Storage, Security, and Management.
- H. All patients with current orders for opioid medications, including buprenorphine, or who are being treated for opioid use disorder shall receive naloxone, or a prescription for naloxone, upon discharge from OSH, per ORS 441.052.
- I. OSH follows all applicable regulations, including federal and state statutes and rules; Oregon Department of Administrative Services (DAS), Shared Services, and Oregon Health Authority (OHA) policies; and relevant accreditation standards. Such regulations supersede the provisions of this policy unless this policy is more restrictive.
- J. Staff who fail to comply with this policy or related policy attachments or protocols may be subject to disciplinary action, up to and including dismissal.

III. DEFINITIONS

"Secure perimeter" means restricted high-security buildings, areas, and quads within the sallyport exits that are operated to manage movement of persons within the OSH campus. The secure perimeter areas include all quads, the ball field, and Medical Clinic.

"Staff" includes employees, volunteers, trainees, interns, contractors, vendors, and other state employees assigned to work at OSH.

"Opioid-related overdose" occurs when a person takes too much of an opioid, or combination of opioids and other drugs, at a level that is toxic to the body. During an opioid related overdose, the part of the brain that regulates breathing is negatively affected, resulting in ineffective or complete cessation of breathing.

"Standing order" means an order that becomes active without the use of a Pharmacy and Therapeutics Committee-approved protocol or without prior review of ordering practitioner.

IV. ATTACHMENTS

Attachment A Standing Order for the Possession, Distribution, and Administration of Naloxone

V. RELATED OSH POLICIES AND PROTOCOLS AND CLINICAL PRACTICE GUIDELINES

Medical Department Publication "MAT for OUD"

1.003 Incident Reporting

8.002 Emergency Care Provided by OSH

8.038 Code Blue Medical Emergency

9.002 Training for Staff

VI. RELATED OSH FORMS

Informed Consent and Management Agreement for Long Term Opioid Use [76050, Medical Record Form]

VII. REFERENCES

Oregon Revised Statute § 441.052

S.B. 1043-B, 82nd Legislative Assembly, 2023 Reg. Sess. (Ore. 2023)